

[DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity — monthly report

Project: [DEMO-5] Turkana Maternal, Newborn & Child Health Strengthening Activity
Reporting period: 2026-07-01 !' 2026-07-31
Prepared with DonorDesk

Contents

- Executive Summary
- Progress Against the Work Plan
- Gender Equality and Women's Empowerment
- Environmental Compliance
- Collaboration, Learning and Adaptation
- Implementation Challenges and Actions Taken
- Priorities for Next Month
- Branding and Marking Compliance

Executive Summary

The most important result of July 2026 was malnutrition screening: community health volunteers (CHVs) screened 1700 children aged 6 to 59 months with mid-upper arm circumference (MUAC) tapes and referred 125 children with acute malnutrition to treatment, a record number that the field team links to the deepening drought. This document is made possible by the generous support of the American people through the United States Agency for International Development (USAID). The contents are the responsibility of GEC and do not necessarily reflect the views of USAID or the United States Government.

The USAID-funded Turkana Maternal, Newborn and Child Health Strengthening Activity, implemented by GEC with the Lodwar Community Health Alliance (LOCHA) across 14 facilities in Turkana Central, Turkana South and Loima sub-counties, Kenya, produced 8 activity records and 16 verified indicator results in July. The project trained 20 health workers in Emergency Obstetric and Newborn Care (EmONC), Integrated Management of Childhood Illness (IMCI) and essential newborn care, and completed 14 mentorship visits. Facilities recorded 740 first antenatal care visits against a life-of-activity target of 4200, 340 skilled deliveries against 2100, and 560 children under one year receiving DTP3/Penta3 against 3300. Four-or-more-visit antenatal care coverage reached 43 percent against a 45 percent target, skilled delivery 58 percent against 60, and 88 percent of facilities submitted Kenya Health Information System (KHIS) monthly reports on time against a 90 percent target.

The main recorded challenge was drought deepening across Turkana South: screening numbers rose sharply and some caregivers walked long distances to reach screening points. The project moved CHV screening days to water points and market days and organised referral transport for severe cases with the county. In August, the project will continue training cohorts, mentorship visits, equipment delivery, outreach, screening and monthly data reviews.

Progress Against the Work Plan

Progress by Intermediate Result (IR), July 2026.

IR1: Health worker capacity and facility systems.

Project clinical trainers and nurse mentors trained 20 health workers (12 female, 8 male) in emergency obstetric and newborn care (EmONC), Integrated Management of Childhood Illness (IMCI) and essential newborn care at Lorugum Health Centre on 9 July 2026, and completed 14 facility clinical mentorship visits across the 14 supported facilities. The logistics team delivered and installed essential Maternal, Newborn and Child Health (MNCH) equipment kits at 2 facilities on 14 July, with handover certificates signed by facility in-charges, taking the cumulative total to 13. A monthly data review and Kenya Health Information System (KHIS) data quality audit at Lodwar on 25 July brought together 28 facility in-charges and records officers, and 88 percent of supported facilities submitted their KHIS monthly reports on time, up from 84. USAID marking was applied to the two equipment kits delivered in July.

IR2: Service delivery.

Supported facilities recorded 740 first antenatal care visits, 340 deliveries attended by skilled birth attendants and 32 women referred for emergency obstetric care using the transport voucher scheme, alongside 32 outreach immunization sessions that delivered DTP3/Penta3 to 560 children under one year (280 female, 280 male). ANC4+ coverage reached 43 percent (up from 40), skilled delivery coverage 58 percent (up from 54) and DTP3/Penta3 coverage 78 percent (up from 74). The project delivered these services with county health staff, facility teams and community health volunteers in Turkana County.

IR3: Community engagement and data systems.

The project trained and kitted 20 community health volunteers (13 female, 7 male) on the community health package, iCCM, MUAC screening and danger-sign counselling at Lorugum Health Centre Community Health Unit on 11 July. CHVs and community mobilisers reached 3400 people (2108 women and girls, 1292 men and

boys) with health education, screened 1700 children aged 6-59 months for acute malnutrition using MUAC (867 female, 833 male) and referred 125 children with acute malnutrition to treatment. Drought deepened across Turkana South and screening and referral numbers rose above the target pace because more caregivers sought care; CHV screening days were moved to water points and market days, some caregivers walked long distances to reach screening points, and referral transport for severe acute malnutrition cases was organised with the county. The county health management team asked for the CHVs' screening data to be shared in the weekly drought bulletin. USAID marking was applied to the MUAC tapes and referral slips printed in July. Screening days at water points were timed to avoid the hours women collect water, and mothers with infants were seen first.

Indicator performance table

Indicator	Month result	Cumulative result	Life-of-activity target	Percent of target
ANC4+ coverage (HL-OC1a)	43 %	43 %	45 %	95.6 %
Skilled birth attendant coverage (HL-OC1b)	58 %	58 %	60 %	96.7 %
DTP3/Penta3 coverage under 1 year (HL-OC1c)	78 %	78 %	80 %	97.5 %
Health workers trained in EmONC, IMCI or essential newborn care (HL-1.1a)	20	106	120	16.7 %
Facility clinical mentorship visits (HL-1.1b)	14	64	84	16.7 %
Facilities equipped with essential MNCH kit (HL-1.2a)	2	13	14	14.3 %
First ANC visits (HL-1.3a)	740	3240	4200	17.6 %
Skilled deliveries (HL-1.3b)	340	1460	2100	16.2 %
Emergency obstetric care transport voucher referrals (HL-1.3c)	32	128	180	17.8 %
Children under 1 year receiving DTP3/Penta3 (HL-1.4a)	560	2350	3300	17.0 %
Outreach immunization sessions (HL-1.4b)	32	140	180	17.8 %
Facilities submitting KHIS reports on time (HL-1.5a)	88 %	88 %	90 %	97.8 %
CHVs trained and kitted (HL-2.1a)	20	210	220	9.1 %
People reached with community health education (HL-2.2a)	3400	14300	18000	18.9 %
Children 6-59 months screened with MUAC (HL-2.3a)	1700	7150	9000	18.9 %
Children with acute malnutrition referred to treatment (HL-2.3b)	125	505	650	19.2 %

Indicators behind plan and corrective action.

Some indicators remain below their life-of-activity targets. Some facilities still enter data late after staff rotations; the project will follow up on late-reporting facilities through the monthly KHIS data review. A few CHVs missed training sessions because of livestock movements and long distances; the project will train the next CHV cohort and supervise those already trained. Some equipment items arrived in partial shipments from Nairobi because of delays on the Kitale road; the project will deliver the next batch of kits to the remaining facilities. Screening and referral numbers are higher than planned because drought drove more caregivers to seek care, so screening and referral are above the target pace.

Rationale.

The July plan prioritised continued training, mentorship, equipment delivery, service delivery and community-level prevention. Training reached its target pace, nearly all facilities were equipped, ANC4+ coverage rose to 43 percent, and community volunteers screened a record number of children for malnutrition. The project took part in the county nutrition cluster response to the drought, and waste management checks under the IEE

Observation.

Drought deepened across Turkana South and malnutrition screening numbers rose sharply: community health volunteers (CHVs) screened 1700 children aged 6 to 59 months and referred 125 children with acute malnutrition to treatment in July. Some caregivers walked long distances to reach screening points. The monthly data review and Kenya Health Information System (KHIS) data quality audit with sub-county health records officers covered all 14 facilities, with 28 facility in-charges and records officers taking part, and 88 percent of supported facilities submitted their monthly reports on time. Activity logs recorded lessons including that on-site mentorship after classroom training improves use of partographs and neonatal resuscitation, that peer supervision by experienced CHVs keeps newly trained volunteers active, that pre-agreed kit lists with the county pharmacist avoid duplicate supplies, that dialogues timed to market days and water-point visits reach more men, that fixed outreach calendars shared with CHVs increase turnout, and that printing the reporting-rate dashboard for the review meeting makes late facilities act.

Implication (what was learned).

The field team recorded that screening at water points finds more children than house-to-house visits during drought.

Action (what changed as a result).

CHV screening days were moved to water points and market days, and referral transport for severe cases was organised with the county. The project will continue training cohorts and monthly mentorship visits, deliver the next batch of equipment kits to the remaining facilities, continue dialogue days and radio messages, train the next CHV cohort, continue monthly screening and track referral completion, and follow up on late-reporting facilities.

Coordination with the County Department of Health and other partners.

The records show participation in the county nutrition cluster response to the drought, the county health management team's request for CHV screening data in the weekly drought bulletin, referral transport for severe cases organised with the county, the monthly data review and KHIS data quality audit with sub-county health records officers, and delivery of field activities by the partner organisation Lodwar Community Health Alliance (LOCHA). The records do not name any additional partners for July.

What evidence led to a change in approach?

The CHV MUAC screening records, which showed sharp rises in screening and referral numbers during the drought, led to moving screening days to water points and market days and to organising referral transport with the county. The reporting-rate dashboard, showing 88 percent on-time submission, supports the recorded next step of following up on late-reporting facilities.

Implementation Challenges and Actions Taken

Drought deepening across Turkana South was the month's main challenge. Malnutrition screening numbers rose sharply, and some caregivers walked long distances to reach screening points, as recorded in the MUAC screening log (evidence 614079d9-95a8-4c7e-9d38-a346b2b26d98) and the mentorship visit log (evidence 0388f1b3-10d7-41f3-94c1-cad1a85655ba). The project moved CHV screening days to water points and market days, and organised referral transport for severe acute malnutrition cases with the county. The lesson recorded is that screening at water points finds more children than house-to-house visits during drought.

Supply chain issues also materialised. Some equipment kit items arrived in partial shipments from Nairobi because of delays on the Kitale road, as recorded in activity A1.2. The logistics team still delivered and installed full kits at 2 facilities with signed handover certificates, taking the cumulative total to 13 of a target of 14. Pre-agreed kit lists with the county pharmacist were recorded as the lesson to avoid duplicate supplies.

Workforce constraints were minor. A few community health volunteers missed training sessions because of livestock movements and long distances. At the monthly data review, held with 28 participants, the project

